HealthWise AI
Medical history summary

Printed 23 Oct 2026, 9:19 pm
by Dr. Anjali Sharma

1 PATIENT IDENTIFICATION

Rahul Patil

28 years | male | Blood group not recorded | ID P-0001 | 168 cm, 71 kg (BMI 25.2)
Emergency contact: not recorded | Patient's phone: 9000000001

ALLERGIES
Penicillin

TAKES REGULARLY
12 medicines
Spironolactone, Donepezil, Aspirin, Clopidogrel and 8 more

LONG-TERM ILLNESSES
Diabetes mellitus, Hypertension, Ischaemic heart disease, Alzheimer's disease, Bronchial asthma, Hypothyroidism
Today: Giddiness, Sweating

2 DETAILED MEDICAL HISTORY

PAST HISTORY

A. DIABETES MELLITUS

Diabetes mellitus [CHECKED BY A DOCTOR]

DIAGNOSIS	Type 2
DURATION	5 years
YEAR OF DIAGNOSIS	2021
ON TREATMENT	Yes
OVERALL CONTROL	Fair
BLOOD SUGAR (REPORTED)	fasting 142 mg/dL, after meal 210 mg/dL, HbA1c 7.6% (measured Sep 2026)
LATEST LAB REPORTS	HbA1c 6.8 % (6 Oct 2026); Random Glucose 130 (6 Apr 2026)
COMPLICATIONS	Eye (retinopathy): No Kidney (nephropathy): Not recorded Heart and vessels: No **Nerves (neuropathy): Yes** Foot / non-healing wounds: No **Low sugar episodes / emergency: Yes**
HOSPITAL STAY FOR THIS	Yes; reason: Very high sugar with vomiting; when: March 2023; treatment: IV fluids and insulin infusion; outcome: Recovered, discharged on tablets
OTHER DETAILS	Follows a diabetic diet; walks 30 minutes daily.
MEDICINES FOR THIS	Tab. Metformin 500 mg (500 mg, twice daily after food, Oral); Insulin glargine (10 units, at bedtime, Injection)

B. HYPERTENSION (HIGH BLOOD PRESSURE)

Hypertension [TOLD BY PATIENT]

DIAGNOSIS	Hypertension
DURATION	8 years
YEAR OF DIAGNOSIS	2018
ON TREATMENT	Yes
OVERALL CONTROL	Good
BLOOD PRESSURE (REPORTED)	usual 130/80, recent 138/86 on 4 Sep 2026 (measured Sep 2026)
COMPLICATIONS	Heart disease: No Kidney damage: Not recorded Stroke: No
HOSPITAL STAY FOR THIS	Yes; reason: Hypertensive emergency, BP 210/120; when: 2022; treatment: IV labetalol, observation; outcome: BP controlled
MEDICINES FOR THIS	Tab. Amlodipine 5 mg (5 mg, once daily, Oral); Tab. Telmisartan 40 mg (40 mg, once daily morning, Oral)

C. OTHER CHRONIC ILLNESSES

Ischaemic heart disease - about 23 years (calculated from the year of diagnosis), since 2003, on treatment **[ENTERED EARLIER]**
Control, complications, hospital stays and medicines: not recorded.

Alzheimer's disease - 2 years, on treatment **[ENTERED EARLIER]**
Control, complications, hospital stays and medicines: not recorded.

Bronchial asthma **[TOLD BY PATIENT]**

DIAGNOSIS DETAILS	Mild persistent
DURATION	12 years
YEAR OF DIAGNOSIS	2014
ON TREATMENT	Yes
DISEASE CONTROL	Good
COMPLICATIONS	**Other: None reported**
HOSPITAL STAY FOR THIS	No
CLINICAL NOTES	Triggered by dust and cold air.
MEDICINES FOR THIS	*Not recorded*

Hypothyroidism **[HOSPITAL RECORD]**

DIAGNOSIS DETAILS	*Not recorded*
DURATION	3 years
YEAR OF DIAGNOSIS	*Not recorded*
ON TREATMENT	Yes
DISEASE CONTROL	Not recorded
HOSPITAL STAY FOR THIS	*Not recorded*
MEDICINES FOR THIS	*Not recorded*

D. MEDICATION HISTORY (MEDICINES TAKEN REGULARLY)

#	MEDICINE	DOSAGE, FREQUENCY	ROUTE	FOR	SINCE	PRESCRIBED BY	TAKES IT	SOURCE
1	**Tab. Spironolactone**	12.5 mg, once daily	*Not recorded*	blood pressure	*Not recorded*	*Not recorded*	*Not recorded*	[ENTERED EARLIER]
2	**Tab. Donepezil**	5 mg, once daily	*Not recorded*	memory	*Not recorded*	*Not recorded*	*Not recorded*	[ENTERED EARLIER]
3	**Tab. Aspirin**	75 mg, once daily	*Not recorded*	heart	*Not recorded*	*Not recorded*	*Not recorded*	[ENTERED EARLIER]
4	**Tab. Clopidogrel**	75 mg, once daily	*Not recorded*	heart	*Not recorded*	*Not recorded*	*Not recorded*	[ENTERED EARLIER]
5	**Penicillin V**	250 mg, twice daily	*Not recorded*	infection	*Not recorded*	*Not recorded*	*Not recorded*	[ENTERED EARLIER]
6	**Tab. Atorvastatin**	*Not recorded*	*Not recorded*	-	*Not recorded*	*Not recorded*	*Not recorded*	[ENTERED EARLIER]
7	**Tab. Metformin 500 mg**	500 mg, twice daily after food	Oral	Diabetes mellitus	2021	Dr. Mehta	Sometimes misses doses	[CHECKED BY A DOCTOR]
8	**Insulin glargine**	10 units, at bedtime	Injection	Diabetes mellitus	2023	Dr. Mehta	Takes regularly	[TOLD BY PATIENT]
9	**Tab. Amlodipine 5 mg**	5 mg, once daily	Oral	Hypertension	2018	*Not recorded*	Takes regularly	[TOLD BY PATIENT]
10	**Tab. Telmisartan 40 mg**	40 mg, once daily morning	Oral	Hypertension	2022	*Not recorded*	*Not recorded*	[TOLD BY PATIENT]
11	**Salbutamol inhaler**	2 puffs, when breathless	Inhaled	Asthma	*Not recorded*	*Not recorded*	*Not recorded*	[TOLD BY PATIENT]

#	MEDICINE	DOSAGE, FREQUENCY	ROUTE	FOR	SINCE	PRESCRIBED BY	TAKES IT	SOURCE
12	**Tab. Levothyroxine 50 mcg**	50 mcg, empty stomach, morning	Oral	-	*Not recorded*	*Not recorded*	Not recorded	**[TOLD BY PATIENT]**

3 PREVIOUS HOSPITAL ADMISSION HISTORY

PAST HISTORY

Admission 1: Right leg injury after a road accident

[TOLD BY PATIENT]

2 Aug 2025, 6:10 pm | Sunrise Orthopaedic Centre | Emergency - **Stay: 5 days 16 hours**

A. ADMISSION DETAILS

ADMITTED	2 Aug 2025, 6:10 pm
DISCHARGED	8 Aug 2025, 11:00 am
LENGTH OF STAY	5 days 16 hours
MAIN COMPLAINT	Right leg injury after a road accident
PROVISIONAL DIAGNOSIS	Right femur fracture
FINAL DIAGNOSIS	Closed fracture, right femur shaft (mid-shaft, transverse)
REASON FOR ADMISSION	Fracture needing surgical fixation
REFERRED BY	Casualty, District Hospital

B. CLINICAL HISTORY DURING THE ADMISSION

SYMPTOMS	Severe pain and swelling of the right thigh, unable to stand.
STORY OF THE ILLNESS	*Not recorded*
VITAL SIGNS AT ADMISSION (2 AUG 2025, 18:20)	BP 128/82 mmHg, pulse 98/min, breathing 18/min, temp 98.6 F, SpO2 99%, pain 8/10
EXAMINATION FINDINGS	Deformity and tenderness of right thigh, distal pulses present.
TESTS AND IMAGING	X-ray right femur: transverse mid-shaft fracture. Random sugar 188 mg/dL.

C. TREATMENT AND MEDICINES DURING THE STAY

TREATMENT GIVEN	Splint, analgesia, surgery on day 2, physiotherapy from day 3.
MEDICINES GIVEN	Inj. Diclofenac - 75 mg, twice daily, route IM, 3 days (painkiller) Inj. Cefuroxime - 750 mg, twice daily, route IV, 5 days (antibiotic)
RESPONSE TO TREATMENT	Pain controlled, wound healthy.

D. OPERATIONS AND SURGICAL HISTORY

Operation: Intramedullary nailing (4 Aug 2025) [HOSPITAL RECORD]

REASON	Displaced transverse fracture
SITE	Right femur, mid-shaft
SURGEON, HOSPITAL	Dr. R. Kulkarni, Sunrise Orthopaedic Centre
ANAESTHESIA	Spinal
SURGICAL FINDINGS	Transverse fracture, no comminution.
IMPLANT OR DEVICE	Yes - Rod / nail - Titanium - ID TI-2291-C
POST-OPERATIVE X-RAY	Good alignment and position of the nail.
POST-OP TREATMENT	Antibiotics, DVT prophylaxis, physiotherapy.
POST-OP COMPLICATIONS	None
RECOVERY PROGRESS	Partial weight bearing at 6 weeks.

E. DISCHARGE SUMMARY AND RECOVERY

CONDITION AT DISCHARGE	Stable, walking with a walker.
DISCHARGE MEDICINES	*Not recorded*
INSTRUCTIONS	Do not bear full weight for 6 weeks. Keep wound dry.

WOUND CARE, REHABILITATION	Physiotherapy twice a week. Suture removal on day 14.
FOLLOW-UP DATE	22 Aug 2025
SPECIALIST REVIEW	Orthopaedics
RECOVERY STATUS	Walking with support at 6 weeks.
REMAINING SYMPTOMS	Mild thigh ache on climbing stairs.

Admission 2: Fever and vomiting for 2 days

[HOSPITAL RECORD]

10 Mar 2024, 9:30 am | City General Hospital | Emergency - **Stay: 3 days 5 hours**

A. ADMISSION DETAILS

ADMITTED	10 Mar 2024, 9:30 am
DISCHARGED	13 Mar 2024, 2:45 pm
LENGTH OF STAY	3 days 5 hours
MAIN COMPLAINT	Fever and vomiting for 2 days
OTHER COMPLAINTS	Loose stools, weakness
PROVISIONAL DIAGNOSIS	Acute gastroenteritis
FINAL DIAGNOSIS	Viral gastroenteritis with mild dehydration
REASON FOR ADMISSION	Dehydration needing IV fluids

B. CLINICAL HISTORY DURING THE ADMISSION

SYMPTOMS	Fever, vomiting 6-7 times a day, loose stools
STORY OF THE ILLNESS	Ate outside food two days before. No blood in stool.
ILLNESSES ALREADY PRESENT	Type 2 diabetes, hypertension
VITAL SIGNS AT ADMISSION (10 MAR 2024, 09:45)	BP 100/60 mmHg, pulse 112/min, breathing 20/min, temp 101.8 F, SpO2 97%, pain 3/10
EXAMINATION FINDINGS	Dry tongue, reduced skin turgor, abdomen soft, mild generalised tenderness.
TESTS AND IMAGING	Hb 13.2 g/dL, WBC 11,200, creatinine 1.4 mg/dL (high), stool routine: no pus cells.

C. TREATMENT AND MEDICINES DURING THE STAY

TREATMENT GIVEN	IV fluids, antiemetics, antibiotics for 3 days, diabetes medicines adjusted.
MEDICINES GIVEN	Inj. Ceftriaxone - 1 g, twice daily, route IV, 3 days (antibiotic) Inj. Paracetamol - 1 g, if fever, route IV, 3 days (painkiller) Normal saline - 1 L, every 8 hours, route IV, 2 days (iv fluid)
IV FLUIDS, OXYGEN	Normal saline 3 L over 24 hours. No oxygen needed.
MEDICATION CHANGES	Metformin stopped during the stay, restarted at discharge.
RESPONSE TO TREATMENT	Fever settled in 24 hours; vomiting stopped on day 2.
COMPLICATIONS IN STAY	Mild kidney function rise, recovered.

D. OPERATIONS AND SURGICAL HISTORY

No operation recorded for this admission.

E. DISCHARGE SUMMARY AND RECOVERY

CONDITION AT DISCHARGE	Stable, eating well, creatinine 1.0 mg/dL.
TREATMENT COMPLETED	3 days of IV antibiotics.
DISCHARGE MEDICINES	ORS - after each loose stool, route Oral Tab. Metformin 500 mg - 500 mg, twice daily, route Oral
INSTRUCTIONS	Light diet, plenty of fluids. Return if fever or vomiting returns.
FOLLOW-UP DATE	20 Mar 2024
SPECIALIST REVIEW	Physician review in 1 week
RECOVERY STATUS	Recovered fully by day 7.

Admission 3: Typhoid fever

[TOLD BY PATIENT]

around 2015 | Village clinic - **Stay: Not recorded**

A. ADMISSION DETAILS

ADMITTED	around 2015
DISCHARGED	*Not recorded*
LENGTH OF STAY	Not recorded
MAIN COMPLAINT	*Not recorded*
PROVISIONAL DIAGNOSIS	*Not recorded*
FINAL DIAGNOSIS	Typhoid (as told)
REASON FOR ADMISSION	Typhoid fever

B. CLINICAL HISTORY DURING THE ADMISSION

SYMPTOMS	*Not recorded*
STORY OF THE ILLNESS	*Not recorded*
VITAL SIGNS AT ADMISSION	*Not recorded*
EXAMINATION FINDINGS	*Not recorded*
TESTS AND IMAGING	*Not recorded*

C. TREATMENT AND MEDICINES DURING THE STAY

TREATMENT GIVEN	*Not recorded*
MEDICINES GIVEN	*Not recorded*
RESPONSE TO TREATMENT	*Not recorded*

D. OPERATIONS AND SURGICAL HISTORY

No operation recorded for this admission.

E. DISCHARGE SUMMARY AND RECOVERY

CONDITION AT DISCHARGE	*Not recorded*
DISCHARGE MEDICINES	*Not recorded*
INSTRUCTIONS	*Not recorded*
FOLLOW-UP DATE	*Not recorded*
RECOVERY STATUS	*Not recorded*

OPERATIONS NOT LINKED TO AN ADMISSION

Operation: Appendectomy (2010) [TOLD BY PATIENT]

REASON	*Not recorded*
SURGEON, HOSPITAL	Old city hospital
ANAESTHESIA	*Not recorded*
IMPLANT OR DEVICE	Not documented

4 ALLERGIES AND MEDICATION RECONCILIATION

PAST AND CURRENT

ALLERGIES AND REACTIONS

Penicillin - skin rash

MEDICINES IN THE HISTORY COMPARED WITH OPEN PRESCRIPTIONS OF THIS HOSPITAL

MEDICINE	IN THE HISTORY	ON AN OPEN PRESCRIPTION	CHECK
Tab. Spironolactone	12.5 mg, once daily	-	**Not on a prescription here: confirm**
Tab. Donepezil	5 mg, once daily	-	**Not on a prescription here: confirm**

MEDICINE	IN THE HISTORY	ON AN OPEN PRESCRIPTION	CHECK
Tab. Aspirin	75 mg, once daily	-	**Not on a prescription here: confirm**
Tab. Clopidogrel	75 mg, once daily	-	**Not on a prescription here: confirm**
Penicillin V	250 mg, twice daily	-	**Not on a prescription here: confirm**
Tab. Atorvastatin	yes	-	**Not on a prescription here: confirm**
Tab. Metformin 500 mg	500 mg, twice daily after food, Oral	-	**Not on a prescription here: confirm**
Insulin glargine	10 units, at bedtime, Injection	-	**Not on a prescription here: confirm**
Tab. Amlodipine 5 mg	5 mg, once daily, Oral	-	**Not on a prescription here: confirm**
Tab. Telmisartan 40 mg	40 mg, once daily morning, Oral	-	**Not on a prescription here: confirm**
Salbutamol inhaler	2 puffs, when breathless, Inhaled	-	**Not on a prescription here: confirm**
Tab. Levothyroxine 50 mcg	50 mcg, empty stomach, morning, Oral	-	**Not on a prescription here: confirm**
Medicine A	*Not recorded*	500 mg, Twice daily (RX-00001, 8 Oct 2026)	**Not in history: confirm and add**
Medicine B	*Not recorded*	10 mg, Once daily (RX-00001, 8 Oct 2026)	**Not in history: confirm and add**

5 FAMILY HISTORY AND LIFESTYLE HABITS

PAST HISTORY

TOBACCO / ALCOHOL / DIET	Chews tobacco (50 years), Alcohol occasionally (30 years), Vegetarian
FAMILY HISTORY	Nothing important.

6 CURRENT COMPLAINT AND HISTORY OF THE PRESENT ILLNESS

TODAY

MAIN COMPLAINT	Giddiness (since 1 day)
ALSO HAS	Sweating
DOCTOR'S RECORD	Giddiness (1 day)

7 CURRENT VITAL SIGNS AND LATEST LABORATORY RESULTS

TODAY AND LATEST RESULTS

VITAL SIGNS AT THIS VISIT (22 OCT 2026, 8:58 PM)

PULSE	BP
46 per min **SLOW**	**138/-** mmHg

BLOOD TEST	LATEST RESULT	NORMAL RANGE	REPORT DATE
HbA1c	**6.8 % HIGH**	4.0 - 5.6	6 Oct 2026
LDL	**118 mg/dL HIGH**	< 100	6 Oct 2026
Vitamin D	**27 ng/mL LOW**	30 - 100	6 Oct 2026
HDL	44 mg/dL	> 40	6 Oct 2026
Hemoglobin	14.2 g/dL	13.0 - 17.0	6 Oct 2026
Random Glucose	130	70 - 140	6 Apr 2026
Total Cholesterol	186 mg/dL	< 200	6 Oct 2026

8 DOCTOR'S CLINICAL EXAMINATION

TODAY

22 Oct 2026, 8:58 pm by Dr. Anjali Sharma

CONSCIOUSNESS	Conscious, well oriented to time, place and person
GENERAL SIGNS	No pallor, icterus, cyanosis, clubbing, oedema, lymph nodes
CHEST (BREATHING)	B/L air entry present, no added sounds, chest movements equal
HEART	S1 S2 heard, no added sounds, no murmur
ABDOMEN	Soft, non-tender, no organomegaly, bowel sounds present
NERVOUS SYSTEM	Higher functions normal, power and tone normal, reflexes normal

9 CLINICAL ASSESSMENT AND DIAGNOSIS

TODAY

No diagnosis written yet. The computer never suggests one.

POINTS TO CHECK (MADE BY THE COMPUTER FROM THIS HISTORY - NOT A DIAGNOSIS)

IMPORTANT FIRST

Allergy to Penicillin and Penicillin V is listed. The patient has a recorded allergy to Penicillin (skin rash), and Penicillin V is among the current medicines. Please check this.

More than one blood-thinning medicine. Tab. Aspirin, Tab. Clopidogrel are listed together. The bleeding risk is higher; the doctor may want to confirm that the combination is intended.

Slow pulse (46 per minute). Below 50 per minute. Medicines that slow the heart and the ECG are worth reviewing.

	TOPIC	WHY IT IS LISTED
MEDICINES	**Raises potassium (Tab. Spironolactone)**	These medicines can raise potassium. Potassium and kidney function (RFT) are worth checking.
MEDICINES	**Kidney function and potassium (Tab. Telmisartan 40 mg)**	ACE inhibitors and ARBs can affect kidney function and potassium. Both are worth checking.
MEDICINES	**Lipids and liver (Tab. Atorvastatin)**	A lipid profile and liver function tests (LFT) are commonly reviewed with a statin. Ask about muscle pain.
MEDICINES	**Bleeding risk (antiplatelet) (Tab. Aspirin, Tab. Clopidogrel)**	Antiplatelet medicines raise the chance of bleeding. Ask about black stools, bruising or bleeding.
MEDICINES	**Kidney function (Tab. Metformin 500 mg)**	Kidney function is commonly checked in patients who take metformin.
MEDICINES	**Low blood sugar (Insulin glargine)**	These medicines can cause low blood sugar, a possible cause of giddiness or blackouts. A blood sugar check is worth considering.
MEDICINES	**Thyroid (Tab. Levothyroxine 50 mcg)**	Thyroid function (TSH) is commonly checked with thyroid hormone tablets.
MEDICINES	**Can slow the heart rate (Tab. Donepezil)**	These medicines for memory loss can slow the heart rate. Pulse and ECG are worth checking if there is fainting, dizziness or a slow pulse.
ILLNESSES	**Long-standing high blood pressure (Hypertension)**	Kidney function, urine protein, ECG, lipid profile and an eye check are commonly reviewed.
ILLNESSES	**Diabetes**	HbA1c, kidney function with urine albumin, lipid profile, and eye and foot examination are commonly reviewed.
ILLNESSES	**Heart disease**	ECG, 2D echo and lipid profile are commonly reviewed. Check that the antiplatelet and statin treatment is continuing as intended.
ILLNESSES	**Lung disease (Bronchial asthma)**	Oxygen saturation, spirometry or peak flow and a chest X-ray are commonly reviewed.
ILLNESSES	**Thyroid disease (Hypothyroidism)**	Thyroid function (TSH) is commonly reviewed.
ILLNESSES	**Memory loss (Alzheimer's disease)**	Medicines deserve a review (some slow the heart rate), as does the risk of falls. Check who looks after the patient and who can give the history.
HABITS	**Tobacco use for 50 years**	Chest examination and ECG are commonly reviewed; look inside the mouth in tobacco chewers; a chest X-ray can be considered.
HABITS	**Long-term alcohol use**	Liver function tests and a complete blood count are commonly reviewed; ask about withdrawal if the patient is admitted.
TODAY'S PROBLEM	**Giddiness**	Pulse, blood pressure (lying and standing), blood sugar and ECG are commonly checked.

10 CURRENT TREATMENT AND MEDICATION PLAN

TODAY

RX-00001 - 8 Oct 2026 - Dr. Anjali Sharma (Follow-up of report values)

MEDICINE	DOSAGE	HOW OFTEN	FOR HOW LONG	INSTRUCTIONS
Medicine A	500 mg	Twice daily	5 days	After food
Medicine B	10 mg	Once daily	5 days	

11 FOLLOW-UP PLAN AND DOCTOR'S NOTES

NEXT STEPS

ADVICE 8 OCT 2026	Follow-up monitoring: Repeat the blood tests in three months and keep a regular walking routine. (follow-up: 7 Nov 2026)
NEXT APPOINTMENT	24 Oct 2026, 11:00 am with Dr Aditi
NOTE 8 OCT 2026	Please discuss these results during consultation and follow the recommended follow-up plan. (Dr. Anjali Sharma)

This summary comes from the history entered by the patient and doctors in HealthWise AI. Items marked "Told by patient" have not been checked by a doctor. Please confirm the details with the patient or the family. It does not replace a doctor's examination.

Doctor's name and signature